

Q I'm carrying twins and hoping to have a natural birth. How will my labor differ from having a single baby?

Many twin births have no complications, but your doctor will keep a close eye on you.

Length of labor

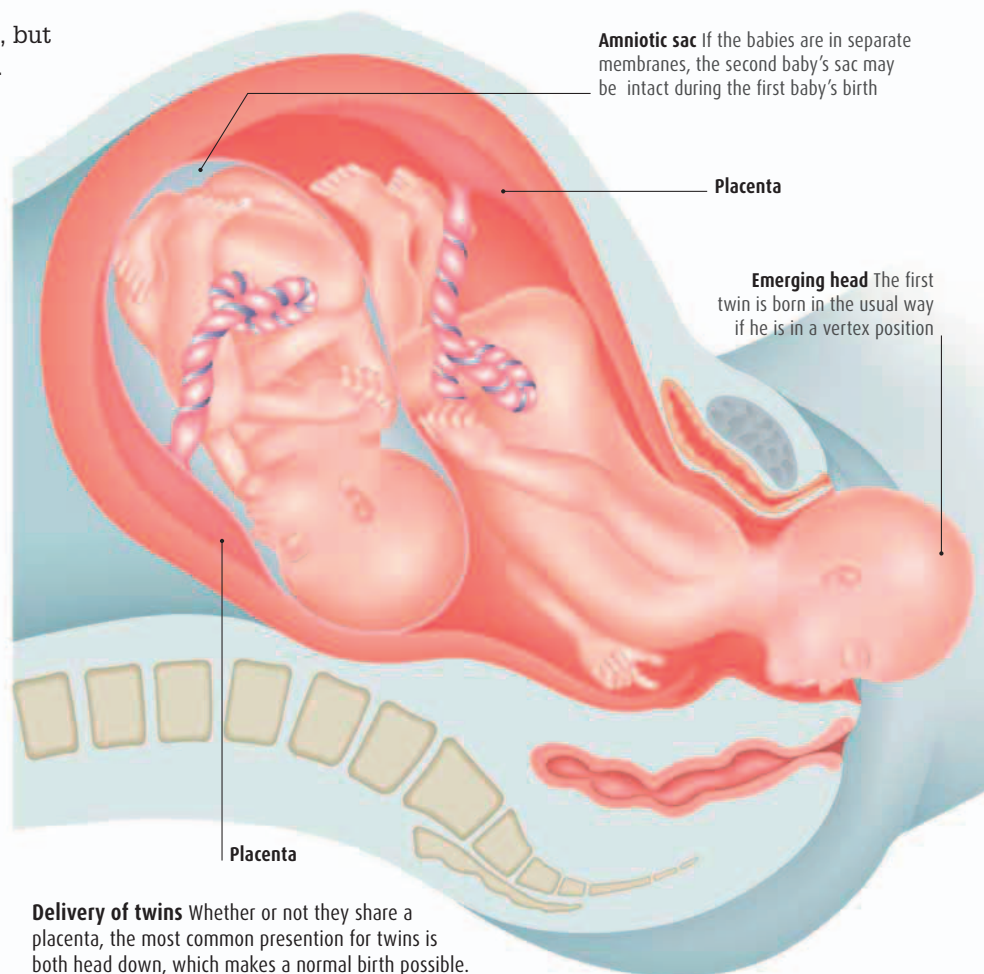
The dilation of the cervix (first stage) takes about the same length of time whether you are having one baby or more. Birthing the babies (second stage) may take longer, because your body has to do the job twice, but the babies are typically smaller than singletons, and can pass through the pelvis more quickly. The overall length of labor is not very different from a singleton birth.

Close monitoring

The heart rates of your babies will be more carefully monitored than if you were giving birth to only one baby. You may initially be able to walk around, especially if your babies are being monitored with handheld devices or unpluggable electrodes on your belly. As your labor progresses closer to the second stage, and usually once your water has broken, you may have continual monitoring that requires you to be stationary. Discuss your hospital's policy on monitoring labor for multiples well in advance of your due date so that you can manage your own expectations of the amount of freedom you're likely to have.

Birthing two babies

If both your babies are positioned head down, or one is head down and the other breech or transverse, it may be possible to deliver first one twin and then the other vaginally. The first baby opens up the birth canal as it is born, and the second baby often turns, or can be turned by palpating your abdomen, after the first is delivered. When your first twin is born you'll be encouraged to bring him to your breast and allow him to nurse (even if you don't intend to breast-feed later on), since this stimulates



Delivery of twins Whether or not they share a placenta, the most common presentation for twins is both head down, which makes a normal birth possible.

your body's natural release of the hormone oxytocin and triggers your uterus to contract again, continuing your second stage labor. Your doctor will try to ensure that your second twin is born within 30 minutes or so of his sibling. If your labor slows down, you may be offered an injection of oxytocin to restart the second stage for the second twin. A long delay between the two births can increase the risk of placental abruption—when your placenta comes away from the uterus lining—for the second baby, which would cut off his blood supply.

Possibility of intervention

On rare occasions—for fewer than five percent of twin births—it is possible that you could need a C-section for the second baby, even if you didn't need one for the first baby. This is more likely if your second twin can't be nudged into a good position for birth, or gets into difficulty, or you become exhausted.

If you ask for pain relief, you are likely to be offered an epidural—not because labor is necessarily more painful with twins, but because analgesics can slow down the babies' heart rates and your medical team won't advise any treatment that could be high risk for multiple births. In addition to this, an epidural works as anesthesia if you suddenly need assisted delivery or a C-section.

In almost 50 percent of twin births both babies present with their head downward—sometimes the first baby is head down, and the second turns once the first is born.